

Acknowledgement

Draft

Antirheumatic • Anti-inflammatory Agent

VRle MYS-002
R.O.C. Reg. No.: 20785

Voren® Injection



Ingredients:

Each 3mL contains:
Diclofenac Sodium 75mg
Benzyl Alcohol 0.15mL

Pharmacodynamics:

Analgesic, antipyretic and anti-inflammatory activities have been demonstrated. It inhibits both prostaglandins synthesis and activity.

Pharmacokinetics:

It is absorbed from GI tract peak plasma concentration occurs about 2 hours after ingestion of enteric coated tablets. More than 99% bound to plasma proteins and is metabolized and excreted mainly in the urine. Small amounts are excreted in the bile.

Indications:

Inflammatory and degenerative forms of rheumatism; rheumatoid arthritis, ankylosing spondylitis; osteoarthritis and spondylarthritis; Non-articular rheumatism; Post-traumatic inflammation and swelling.

Dosage and administration:

As a general recommendation, the dose should be individually adjusted. Adverse effects may be minimized by using the lowest effective dose for the shortest duration necessary to control symptoms (see section Precautions/Warnings).

Established Cardiovascular Disease or Significant Cardiovascular Risk Factors

Treatment with diclofenac is generally not recommended in patients with established cardiovascular disease (congestive heart failure, established ischemic heart disease, peripheral arterial disease) or uncontrolled hypertension. If needed, patients with established cardiovascular disease, uncontrolled hypertension, or significant risk factors for cardiovascular disease (e.g. hypertension, hyperlipidaemia, diabetes melitus and smoking) should be treated with diclofenac only after careful consideration and only at doses ≤ 100mg daily if treated for more than 4 weeks (see section Precautions/Warnings).

IM Injection:

Usually administer 1 ampoule (3mL) for adult, one time daily. For systemic therapy, injection should be made deeply into the gluteal muscle to insure intramuscular delivery. Use alternate sites for subsequent injection. To be administered by a physician.

Route of Administration:

For intramuscular use.

Contraindications:

1. Known hypersensitivity to diclofenac sodium.
2. Active gastric or intestinal ulcer, bleeding or perforation; severe hepatic, renal or severe cardiac failure (see Precautions).
3. Contraindicated in patients in whom attacks of asthma, urticaria, or acute rhinitis are precipitated by acetylsalicylic acid or other NSAIDs.

Precaution(s) / Warning(s):

1. Caution should be exercised in patients with a history of blood dyscrasias or disorders of coagulation.
2. Cardiovascular effects
Treatment with NSAIDs including diclofenac, particularly at high dose and in long term, may be associated with an increased risk of serious cardiovascular thrombotic events (including myocardial infarction and stroke).
Treatment with diclofenac is generally not recommended in patients with established cardiovascular disease (congestive heart failure, established ischemic heart disease, peripheral arterial disease) or uncontrolled hypertension. If needed, patients with established cardiovascular disease, uncontrolled hypertension, or significant risk factors for cardiovascular disease (e.g. hypertension, hyperlipidaemia, diabetes mellitus and smoking) should be treated with diclofenac only after careful consideration and only at doses ≤ 100mg daily when treatment continues for more than 4 weeks.
As the cardiovascular risks of diclofenac may increase with dose and duration of exposure, the lowest effective daily dose should be used for the shortest duration possible. The patient's need for symptomatic relief and response to therapy should be re-evaluated periodically, especially when treatment continues for more than 4 weeks.
Patients should remain alert for the signs and symptoms of serious arteriothrombotic events (e.g. chest pain, shortness of breath, weakness, slurring of speech), which can occur without warnings. Patients should be instructed to see a physician immediately in case of such an event. There is no consistent evidence that the concurrent use of aspirin mitigates the possible increased risk of serious cardiovascular thrombotic events associated with NSAID use.
3. Gastrointestinal effects
NSAIDs, including diclofenac, may be associated with increased risk of gastrointestinal anastomotic leak. Close medical surveillance and caution are recommended when using diclofenac after gastrointestinal surgery.
4. Severe cutaneous reactions, including Stevens - Johnson syndrome and toxic epidermal necrolysis (Lyell's syndrome), have been reported with diclofenac sodium. Patients treated with diclofenac sodium should be closely monitored for signs of hypersensitivity reactions. Discontinue diclofenac sodium immediately if rash occurs.
5. Adverse effects: Dermatological: Occasional - rashes or skin eruptions Cases of hair loss, bullous eruptions, erythema multiforme, Stevens-Johnson syndrome, toxic epidermal necrolysis (Lyell's syndrome), and photosensitivity reactions have been reported.
6. NSAIDs may lead to the onset of new hypertension or worsening the pre-existing hypertension and patients taking anti-hypertensive with NSAIDs may have an impaired anti-hypertensive response. Caution is advised when prescribing NSAIDs to patients with hypertension. Blood pressure should be monitored closely during initiation of NSAID treatment and at regular intervals thereafter.
7. Fluid retention and oedema have been observed in some patients taking NSAIDs, there caution is advised in patients with fluid retention or heart failure.
8. All NSAIDs can cause gastrointestinal discomfort and rarely serious, potentially fatal gastrointestinal effects such as ulcers, bleeding and perforation which may increase with dose or duration of use, but can occur at any time without warning. Caution is advised in patients with risk factors for gastrointestinal events e.g. the elderly, those with a history of serious gastrointestinal events, smoking and alcoholism. When gastrointestinal bleeding or ulceration occurs in patients receiving NSAIDs, the drug should be withdrawn immediately. Doctors should warn patients about signs and symptoms of serious gastrointestinal toxicity. The concurrent use of aspirin and NSAIDs also increases the risk of serious gastrointestinal adverse events.
9. NSAIDs may very rarely cause serious cutaneous adverse events such as exfoliative dermatitis, toxic epidermal necrolysis (TEN) and Stevens-Johnson Syndrome (SJS), which can be fatal and occur without warning. These serious adverse events are idiosyncratic and are independent of dose or duration of use. Patients should be advised of the signs and symptoms of serious skin reaction and to consult their doctor at the first appearance of a skin rash or any other sign of hypersensitivity.
10. The anti-inflammatory, antipyretic, and analgesic effects of diclofenac sodium may mask the normal signs of infections. The physician should be alert to any development of infection in patients receiving diclofenac sodium.
11. Patients with severe hepatic, cardiac or renal insufficiency or the elderly should be kept under close surveillance.
12. Use of Diclofenac in patients with hepatic porphyria may trigger an attack.

WARNINGS

RISK OF GI ULCERATION, BLEEDING AND PERFORATION WITH NSAID

Serious GI toxicity such as bleeding, ulceration and perforation can occur at any time, with or without warning symptoms, in patients treated with NSAID therapy. Although minor upper GI problems (eg. dyspepsia) are common, usually developing early in therapy, prescribers should remain alert for ulceration and bleeding in patients treated with NSAIDs even in the absence of previous GI tract symptoms. Studies to date have not identified any subset of patients not at risk of developing peptic ulceration and bleeding. Patients with prior history of serious GI events and other risk factors associated with peptic ulcer disease (eg. alcoholism, smoking and corticosteroid therapy) are at increased risk. Elderly or debilitated patients seem to tolerate ulceration or bleeding less than other individuals and account for most spontaneous reports for fatal GI events.

As this preparation contains benzyl alcohol, its use should be avoided in children under two years of age. Not to be used in neonates.

Interactions with Other Medicaments:

1. **Lithium:** Diclofenac may raise plasma concentrations of lithium. Monitoring of the serum lithium level is recommended.
2. **Digoxin:** Diclofenac may raise plasma concentrations of digoxin. Monitoring of the serum digoxin level is recommended.
3. **Diuretics and Antihypertensive Agents [eg. β -blockers, angiotensin converting enzyme (ACE) inhibitors]:** May cause a decrease in their antihypertensive effect. Therefore, the combination should be administered with caution and patients, especially the elderly should have their blood pressure periodically monitored. Patients should be adequately hydrated and consideration should be given to monitoring of renal function after initiation of concomitant therapy and periodically thereafter, particularly for diuretics and ACE inhibitors due to the increased risk of nephrotoxicity. Concomitant treatment with potassium-sparing drugs may be associated with increased serum potassium levels, which should therefore be monitored frequently.
4. **Other NSAIDs and Corticosteroids:** May increase the frequency of gastrointestinal undesirable effects.
5. **Anticoagulants and Anti-platelet Agents:** Caution is recommended since the risk of bleeding could increase. Close monitoring of such patients is therefore recommended.
6. **Selective Serotonin Reuptake Inhibitors (SSRIs):** May increase the risk of gastrointestinal bleeding.
7. **Antidiabetics:** There have been isolated reports of both hypoglycaemic and hyperglycaemic effects necessitating changes in the dosage of the antidiabetic agents during treatment with diclofenac. Monitoring of the blood glucose level is recommended as a precautionary measure during concomitant therapy.
8. **Methotrexate:** Caution is recommended when NSAIDs are administered < 24 hrs before or after treatment with methotrexate, since blood concentrations of methotrexate may rise and the toxicity of this substance be increased.
9. **Ciclosporin:** Diclofenac may increase the nephrotoxicity of ciclosporin due to the effect on renal prostaglandins. Therefore, it should be given at doses lower than those that would be used in patients not receiving ciclosporin.
10. **Quinolone Antibacterials:** There have been isolated reports of convulsions which may have been due to concomitant use of quinolones and NSAIDs.

Pregnancy and Lactation:

1. This drug should not be used during the first 2 trimesters of pregnancy unless the potential benefit to the mother outweighs the risk to the foetus. As with other NSAIDs, use during the 3rd trimester of pregnancy is contraindicated owing to the possibility of uterine inertia and/or premature closure of the ductus arteriosus.
2. Impairment of Fertility: The use of diclofenac sodium may impair female fertility and is not recommended in women attempting to conceive. In women who have difficulties conceiving or who are undergoing investigation of infertility, withdrawal of diclofenac sodium should be considered.
3. Diclofenac passes into the breast milk in small amounts. Therefore, diclofenac sodium should not be administered during breastfeeding in order to avoid undesirable effect in the infant.

Side Effect(s):

Infections and Infestations: Injection site abscess.

Blood and Lymphatic System Disorders: Thrombocytopenia, leukopenia, anaemia (including haemolytic and aplastic anaemia), agranulocytosis.

Immune System Disorders: Hypersensitivity, anaphylactic, anaphylactoid reactions (including hypotension and shock), angioneurotic oedema (including face oedema).

Psychiatric Disorders: Disorientation, depression, insomnia, nightmare, irritability, psychotic disorder.

Nervous System Disorders: Headache, dizziness, somnolence, paraesthesia, memory impairment, convulsion, anxiety, tremor, aseptic meningitis, taste disturbances, cerebrovascular accident.

Eye Disorders: Visual disturbances, blurred vision, diplopia.

Ear and Labyrinth Disorders: Vertigo, tinnitus, impaired hearing.

Cardiac Disorders:

Uncommon*: Myocardial infarction, cardiac failure, palpitations, chest pain. *The frequency reflects data from long-term treatment with a high dose (150mg/day).

Not known: Kounis syndrome

Vascular Disorders: Hypertension, vasculitis.

Respiratory, Thoracic and Mediastinal Disorders: Asthma (including dyspnoea), pneumonitis.

Gastrointestinal Disorders: Nausea, vomiting, diarrhoea, dyspepsia, abdominal pain, flatulence, anorexia, gastritis, gastrointestinal haemorrhage, melena, haematemesis, haemorrhagic diarrhoea, gastrointestinal ulcer (with or without bleeding or perforation), colitis (including haemorrhagic colitis and exacerbation of ulcerative colitis or Crohn's disease), constipation, stomatitis, glossitis, oesophageal disorder, diaphragm-like intestinal strictures, pancreatitis.

Hepatobiliary Disorders: Increased transaminases, hepatitis, jaundice, liver disorder, fulminant hepatitis.

Skin and Subcutaneous Tissue Disorders: Rash, urticaria, bullous eruptions, eczema, erythema, erythema multiforme, Stevens-Johnson syndrome, toxic epidermal necrolysis (Lyell's syndrome), exfoliative dermatitis, loss of hair, photosensitivity reaction, purpura, allergic purpura, pruritus.

Renal and Urinary Disorders: Acute renal failure, haematuria, proteinuria, nephrotic syndrome, interstitial nephritis, renal papillary necrosis.

General Disorders and Administration Site Conditions: Injection site reaction, injection site pain, injection site induration, oedema, injection site necrosis.

Arteriothrombotic events: Meta-analysis and pharmacoepidemiological data point towards an increased risk of arteriothrombotic events (for example myocardial infarction) associated with the use of diclofenac, particularly at a high dose (150mg daily) and during long-term treatment (see section WARNINGS AND PRECAUTIONS).

Symptoms and Treatment for Overdosage, and Antidote(s):

Symptoms: Overdosage can cause symptoms eg, vomiting, gastrointestinal haemorrhage, diarrhoea, dizziness, tinnitus or convulsions. In the event of significant poisoning, acute renal failure and liver damage are possible.

Treatment: Management of acute poisoning with nonsteroidal anti-inflammatory drugs (NSAIDs) essentially consists of supportive measures and symptomatic treatment. Supportive measures and symptomatic treatment should be given for complications eg, hypotension, renal failure, convulsions, gastrointestinal disorder and respiratory depression.

Special measures eg, forced diuresis, dialysis or haemoperfusion are probably of no help in eliminating NSAIDs due to the high protein binding and extensive metabolism.

Incompatibilities:

Diclofenac sodium injection for intramuscular or intravenous use should not be mixed with other solutions for injections, other than those for diluting as directed.

Shelf-life:

5 years from the date of manufacture.

Storage conditions:

Store at room temperature not exceeding 30°C.

Product Description:

A clear, colorless to light yellow color solution.

Packaging:

3mL/Amp. x 10 Amps.,
3mL/Amp. x 100 Amps.

Manufactured by:

YUNG SHIN PHARMACEUTICAL IND. CO., LTD.

Taichung Youth Factory

No. 27, Kung Chiu Rd., Tachia, Taichung,

Taiwan, R.O.C.

Importer and Product Registration Holder:

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